

STEVEN C. BAST, M.D., Q.M.E

QUALIFIED MEDICAL EVALUATOR

PANEL QUALIFIED MEDICAL EVALUATION IN THE SPECIALTY OF ORTHOPAEDIC SURGERY

+Today's date

+Adjuster Name and Address

+Defense Attorney Name and Address

+Applicant Attorney Name and Address

Re:	+Applicant Name
Employer:	+
WCAB No.:	+
Applicant DOB:	+
Dates/Dates of Injury:	+
Claim/File No.:	+
Date of Evaluation:	+
Place of Evaluation:	+
Interpreter name and #:	+

RE: +Applicant last name, first name

+Date of Report

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Dear Parties:

Pursuant to your authorization, + underwent a Panel Qualified +Evaluation, in the specialty of + on + at my + office. The undersigned acted in the capacity of Panel Qualified Evaluator, in the specialty of Orthopaedic surgery.

Dr. Bast conducted the interview, reviewed all records, performed a physical examination, and formulated the diagnosis, conclusions, and discussion, including the opinion on causation, temporary disability, permanent disability, degree of disability, future care, work restrictions, and apportionment. The report was authored by Dr. Bast. All opinions expressed herein are solely the opinions of Dr. Bast.

Prior to the evaluation, the entire medical file available to this physician was fully reviewed. All of the records reviewed were instrumental in this evaluator arriving at the opinions as expressed in this report. The new medical legal fee schedule, which went into effect on April 1, 2021, requires that all medical records submitted to the QME be accompanied by a declaration.

“Any documents sent to the physician for record review must be accompanied by a declaration under penalty of perjury that the provider of the documents has complied with the provisions of Labor Code section 4062.3 before providing the documents to the physician. The declaration must also contain an attestation as to the total page count of the documents provided. A physician may not bill for review of documents that are not provided with this accompanying required declaration from the document provider. Any documents or records that are sent to the physician without the required declaration and attestation shall not be considered available to the physician or received by the physician for purposes of any regulatory or statutory duty of the physician regarding records and report writing.”

Accordingly, if no declaration was received from a submitting party then the accompanying documents or records were not considered available and therefore were not reviewed. If the parties wish for this QME to review any records or documents which were not previously submitted with the required declaration then please email this to Bastteam@ortholegalgroup.com, along with a letter requesting a supplemental report.

Before I began the examination, the applicant was informed that this evaluation was being done exclusively in connection with the Workers’ Compensation claim at the request of an attorney, attorneys or insurance companies, and that no treatment relationship existed. The applicant was also made aware that any communication between us is not privileged (no doctor-patient confidentiality exists) and that any information provided, as well as the results of any testing and my conclusions regarding the case, would be included in a report that may be read by people involved in the resolution and/or litigation of the claim. The applicant was advised of his or her rights pursuant to QME regulation 40. The applicant stated that the aforementioned was understood, and agreed to proceed with the evaluation. The report belongs to the party or parties requesting the evaluation.

Time spent includes:

- | | |
|--|------------------------|
| 1. Face-to-face interview with the applicant | 0.50/0.75 hours |
|--|------------------------|

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Record Sender Name	Record Sender Company	Declaration – Page Count Attestation	Physician Record Receipt Date	Number of pages of records reviewed by the physician as part of the medical-legal evaluation and preparation of the report

This report has been prepared pursuant to the provisions of 8 Cal. Code Regulations §9795 as a ML 201-95-** / ML202-95-** Basic / Comprehensive Medical-Legal Evaluation conducted by a Qualified Medical Evaluator.

☒ MLPRR – Record Review <u>Billable units:</u> Page count in excess of 200 pages <u>Reimbursement:</u> \$3.00 per unit (page) <u>Under penalty of perjury:</u> ☒ Total pages reviewed by physician evaluator: ** less 200 = ** billable units ☒ §4062.3 Declaration and page count attestation received. Attestation page count: ** ☒ §9795 Verification. Record Review report includes verification under penalty of perjury of the total number of pages of records reviewed by physician. ☐ Description of Circumstances Requiring -93 modifier: An interpreter was needed because the injured worker spoke a language different from the language spoken by the evaluator. These circumstances dictate that when the injured worker spoke words, the evaluator could not understand the injured worker’s words, and vice-versa. ☐ Increased time required due to presence of interpreter: The need for an interpreter increased the time necessary for the evaluation by the amount of time it takes to communicate in two separate languages as enabled by the presence of an interpreter.

REVIEW OF RECORDS PROVIDED

+

****KEEP LANGUAGE HERE IF NO DECLARATION RECEIVED****

Medical records were not available to this physician. This may be because they were not sent or because they were sent but were not accompanied by a required declaration. The new medical legal fee schedule, which went into effect on April 1, 2021, requires that all medical records submitted to the QME be accompanied by a declaration.

Per 8 CCR 9793(n),

“Any documents sent to the physician for record review must be accompanied by a declaration under penalty of perjury that the provider of the documents has complied with the provisions of Labor Code section 4062.3 before providing the documents to the physician. The declaration must also contain an attestation as to the total page count of the documents provided. A physician may

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Accordingly, if no declaration was received from a submitting party then the accompanying documents or records were not considered available and therefore were not reviewed. If the parties wish for this QME to review any records or documents which were not previously submitted with the required declaration then please email this to Bastteam@ortholegalgroup.com, along with a letter requesting a supplemental report.

IDENTIFYING DATA

Mr./Ms. + is a +-year-old +-hand dominant +Caucasian/African American/Hispanic/Asian man/woman who resides in _____, California. + date of birth is +. + is evaluated today pursuant to a claim of (specific injury/cumulative trauma) on +, to the +, while + was working for +, as a +.

HISTORY OF INJURY AS DESCRIBED BY INJURED WORKER

The applicant states that on +

JOB DESCRIPTION:

+

CURRENT CONDITION

+

CURRENT EMPLOYMENT

(same employer or new employer? Working full or modified duties? Collecting any benefits?)

CURRENT TREATMENT

The applicant is currently seeing +

OR

The applicant is currently not receiving any treatment.

PRIOR INJURIES:

Industrial injuries:

•

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Nonindustrial injuries:

•

OCCUPATIONAL HISTORY

+ (Minimum 10 years of employment history)

ACTIVITIES OF DAILY LIVING

	Without Difficulty	With Some Difficulty	With Moderate Difficulty	Unable To Do
Personal Hygiene - Can You:				
Take a bath				
Brush your teeth				
Dress yourself				
Comb your hair				
Eat and drink				
Have a bowel movement				
Urinate				
Communication - Can You:				
Write comfortably				
Type on a computer				
See clearly (with glasses if needed)				
Hear clearly				
Speak clearly				
Physical Activity - Can You:				
Stand				
Sit				
Walk				
Climb stairs				
Sensory Function - Can You:				
Feel what you touch				
Smell the food you eat				
Taste the food you eat				
Hand Function - Can You:				
Hold something without pain				
Open windows at home				
Lift a child				

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Travel - Can You:				
Ride in a car for 30 minutes				
Fly in a plane				
Sexual Function - Can You:				
Have sexual intercourse				
Sleep Restfully				

PAST MEDICAL HISTORY

•

OR

The applicant denies any past medical history.

PAST SURGICAL HISTORY

•

CURRENT MEDICATIONS

•

The applicant last took +

OR

The applicant is presently not taking any medications.

ALLERGIES

No known drug allergies

FAMILY HISTORY

•

OR

The applicant denies family history of disease.

SOCIAL HISTORY

+The applicant denies smoking, alcohol use, and recreational drug use.

The applicant's hobbies include (especially include any sports or physical activity)

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OR

The applicant reports +

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PHYSICAL EXAMINATION

GENERAL

Height (in)	
Weight (lbs)	
BMI	#DIV/0!

VITALS

Temperature	
Heart Rate	
Respiratory Rate	
Blood Pressure	

CERVICAL SPINE

Normal alignment without visible deformity, no spasms observed, skin intact, no surgical scars.

Moderate tenderness to palpation about occiput, C1 – T1 spinous processes.

Tender to palpation over paraspinal musculature, and trapezius bilaterally.

Moderate guarding and asymmetry of spinal motion noted.

CERVICAL SPINE FLEXION

	1 st Measure	2 nd Measure	3 rd Measure	Average
Calvarium Angle				
T1 Angle				
Flexion Angle				

CERVICAL SPINE EXTENSION

	1 st Measure	2 nd Measure	3 rd Measure	Average
Calvarium Angle				
T1 Angle				
Extension Angle				

CERVICAL RIGHT LATERAL BENDING

	1 st Measure	2 nd Measure	3 rd Measure	Average
Calvarium Angle				
T1 Angle				
Right Lateral Bending Angle				

CERVICAL LEFT LATERAL BENDING

	1 st Measure	2 nd Measure	3 rd Measure	Average
Calvarium Angle				
T1 Angle				
Left Lateral Bending Angle				

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CERVICAL ROTATION

	1 st Measure	2 nd Measure	3 rd Measure	Average
Right Rotation Angle				
Left Rotation Angle				

Spine range of motion values obtained after appropriate warm-up and with adequate motivational instruction. The applicant was compliant and put forth a strong effort to achieve the highest possible motion for measurement. Two inclinometers were utilized for measurements and the values recorded above.

GENERALIZED MUSCLE TONE

Upper and lower extremity musculature with normal tone. No evidence of spasticity or fasciculations. There is no noted atrophy of the shoulder, upper arm, forearm or hand compared to opposite extremity.

CIRCUMFERENTIAL MEASUREMENTS:

	Right	Left
Mid Upper arm		
Mid Forearm		

CERVICAL SPINE SENSORY AND MOTOR

	SENSORY		MOTOR	
	RIGHT	LEFT	RIGHT	LEFT
C4	Intact	Intact	5	5
C5	Intact	Intact	5	5
C6	Intact	Intact	5	5
C7	Intact	Intact	5	5
C8	Intact	Intact	5	5
T1	Intact	Intact	5	5

CERVICAL SPINE REFLEXES

	RIGHT	LEFT
C5 (biceps)	2+	2+
C6 (brachioradialis)	2+	2+
C7 (triceps)	2+	2+

CERVICAL SPINE SPECIALIZED NEUROLOGIC TESTS

	RIGHT	LEFT
Spurling's Test	Negative	Negative
Wartenberg's Sign	Negative	Negative
Hoffman's Sign	Negative	Negative
Pain with axial load	Negative	

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THORACIC SPINE

Normal alignment without visible deformity, skin intact, no surgical scars.

Minimal tenderness to palpation about T1 – T12 spinous processes

Minimal tenderness paraspinal musculature bilaterally.

Minimal guarding and asymmetry of spinal motion noted.

THORACIC SPINE FLEXION

	1 st Measure	2 nd Measure	3 rd Measure	Average
T1 Angle				
T12 Angle				
Flexion Angle				

THORACIC SPINE EXTENSION

	1 st Measure	2 nd Measure	3 rd Measure	Average
T1 Angle				
T12 Angle				
Extension Angle				

THORACIC RIGHT LATERAL ROTATION

	1 st Measure	2 nd Measure	3 rd Measure	Average
T1 Angle				
T12 Angle				
Right Lateral Rotation Angle				

THORACIC LEFT LATERAL ROTATION

	1 st Measure	2 nd Measure	3 rd Measure	Average
T1 Angle				
T12 Angle				
Left Lateral Rotation Angle				

Spine range of motion values obtained after appropriate warm-up and with adequate motivational instruction. The applicant was compliant and put forth a strong effort to achieve the highest possible motion for measurement. Two inclinometers were utilized for measurements and the values recorded above.

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LUMBAR SPINE

Normal alignment without visible deformity, skin intact, no surgical scars.

No Scoliotic deformity noted

Moderate tenderness to palpation about L1 – L5 spinous processes.

Non-tender to the paraspinal musculature bilaterally.

Moderate guarding and asymmetry of spinal motion noted.

LUMBAR SPINE FLEXION

	1 st Measure	2 nd Measure	3 rd Measure	Average
T12 Angle				
Sacral Angle				
Flexion Angle				

LUMBAR SPINE EXTENSION

	1 st Measure	2 nd Measure	3 rd Measure	Average
T12 Angle				
Sacral Angle				
Extension Angle				

LUMBAR RIGHT LATERAL BENDING

	1 st Measure	2 nd Measure	3 rd Measure	Average
T12 Angle				
Sacral Angle				
Right Lateral Bending Angle				

LUMBAR LEFT LATERAL BENDING

	1 st Measure	2 nd Measure	3 rd Measure	Average
T12 Angle				
Sacral Angle				
Left Lateral Bending Angle				

Spine range of motion values obtained after appropriate warm-up and with adequate motivational instruction. The applicant was compliant and put forth a strong effort to achieve the highest possible motion for measurement.

Two inclinometers were utilized for measurements and the values recorded above.

TENSION SIGNS:

	RIGHT	LEFT
Seated Sciatic Tension Sign: with the hip flexed to 90°	Negative for back pain with knee flexed to 0°; No radicular component	Negative for back pain with knee flexed to 0°; No radicular component
Supine Sciatic Tension Sign	Positive for back pain with the hip flexed to 65° No radicular component	Positive for back pain with the hip flexed to 65° No radicular component.

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HIP FLEXION VALIDATION TEST

	RIGHT	LEFT
FLEXION (with knee extended)		

LUMBAR SPINE SENSORY AND MOTOR

	SENSORY		MOTOR	
	RIGHT	LEFT	RIGHT	LEFT
L2	Intact	Intact	5	5
L3	Intact	Intact	5	5
L4	Intact	Intact	5	5
L5	Intact	Intact	5	5
S1	Intact	Intact	5	5

LUMBAR SPINE REFLEXES

	RIGHT	LEFT
L4 (patellar)	2+	2+
S1 (achilles)	2+	2+

GENERALIZED MUSCLE TONE

Upper and lower extremity musculature with normal tone. No evidence of spasticity or fasciculations.

GIRTH MEASUREMENTS:

	RIGHT (cm)	LEFT (cm)
Arm		
Forearm		
Thigh		
Calf		

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UPPER EXTREMITIES

SHOULDER

Cervical spine shows full range of motion with negative compression test.

Right shoulder

No redness, swelling, or bruising

No surgical scars

No noted atrophy or hypertrophy.

Non tender to palpation at the acromioclavicular joint, clavicle or sternoclavicular joint.

Non tender to the periscapular region with no periscapular popping or snapping

Tenderness to palpation about **long head of biceps tendon, lateral subdeltoid bursa, and posterior glenohumeral joint.**

“Popeye” sign (tear of long head bicep tendon)

Negative Neer’s and Hawkin’s signs for impingement.

Negative/Positive O’Brien’s test

Negative/Positive apprehension sign

Negative/Positive relocation test.

Full strength in rotator cuff musculature.

Left shoulder

No redness, swelling, or bruising

No surgical scars

No noted atrophy or hypertrophy.

Non tender to palpation at the acromioclavicular joint, clavicle or sternoclavicular joint.

Non tender to the periscapular region with no periscapular popping or snapping

Tenderness to palpation about **long head of biceps tendon, lateral subdeltoid bursa, and posterior glenohumeral joint.**

“Popeye” sign (tear of long head bicep tendon)

Negative Neer’s and Hawkin’s signs for impingement.

Negative/Positive O’Brien’s test

Negative/Positive apprehension sign

Negative/Positive relocation test.

Full strength in rotator cuff musculature.

STRENGTH: (0-5, Normal 5)

	RIGHT	LEFT
Forward flexion		
Scapular abduction		
External rotation		
Internal rotation		

SHOULDER RANGE OF MOTION

	RIGHT	LEFT
FLEXION (N = 180)		
EXTENSION (N = 50)		
ABDUCTION (N = 170)		
ADDUCTION (N = 40)		

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EXTERNAL ROTATION (N = 60)		
INTERNAL ROTATION (N = 80)		

No generalized ligamentous laxity (**Beighton score**)

No noted scapular winging.

Crepitus absent/ present with range of motion at the glenohumeral joint.

Radial Pulse 2+ 1+ Absent

Adson sign Positive / Negative

Sensation is intact to light touch

DTR's REFLEXES

	RIGHT	LEFT
Biceps	2+	2+
Brachioradialis	2+	2+
Triceps	2+	2+

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ELBOW

Cervical spine shows full range of motion with negative compression test.

Right elbow

No surgical scars

No atrophy or hypertrophy noted

No tenderness to palpation extensor wad

Cozen test: **Positive**

Positive tenderness to palpation flexor wad, pain with resisted pronation

No tenderness to palpation flexor wad

Negative Tinel's, compression, and scarecrow at the cubital tunnel

Negative hyperflexion test

No pain elicited along the course of the biceps tendon with resisted flexion and supination.

Negative Hook sign of the Biceps tendon

"Popeye" sign

Ligamentous exam negative for instability; negative milking sign

Distal sensation is intact to light touch

DTR's 2+ at Biceps, Brachioradialis, and Triceps

Radial pulse 2+

Left elbow

No surgical scars

No atrophy or hypertrophy noted.

No tenderness to palpation extensor wad

No tenderness to palpation flexor wad

Cozen test: **Positive**

Positive tenderness to palpation flexor wad, pain with resisted pronation

Negative Tinel's, compression, and scarecrow at the cubital tunnel

Negative hyperflexion test

No pain elicited along the course of the biceps tendon with resisted flexion and supination.

Negative Hook sign of the Biceps tendon

"Popeye" sign

Ligamentous exam negative for instability; negative milking sign

Distal sensation is intact to light touch

DTR's 2+ at Biceps, Brachioradialis, and Triceps

Radial pulse 2+

CIRCUMFERENTIAL MEASUREMENTS:

	Right	left
Mid Upper arm		
Mid Forearm		

ELBOW RANGE OF MOTION

	RIGHT	LEFT
EXTENSION (N = 0)		
FLEXION (N = 140)		

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FOREARM RANGE OF MOTION

	RIGHT	LEFT
SUPINATION (N = 70)		
PRONATION (N = 80)		

WRIST / HAND

Cervical spine shows full range of motion with negative compression test.

Right wrist

Skin without redness, swelling, or bruising.

No surgical scars.

No Thenar or hypothenar atrophy

Tenderness to palpation about **dorsal and volar aspect** of joint.

Ligamentous examination **negative** for instability.

Negative Phalen's, Durkin's, and Tinel's for carpal tunnel

No signs of triggering of the thumb, index finger, middle finger, ring finger, or small finger, at the A1 pulley.

Positive Finkelstein's test

Positive thumb grind / distraction test at CMC joint

Sensation intact to light touch to the first dorsal web space of the hand, volar pad of the index finger, volar pad of the small finger.

Allen test positive

Left wrist

Skin without redness, swelling, or bruising.

No surgical scars.

No Thenar or hypothenar atrophy

Tenderness to palpation about **dorsal and volar aspect** of joint.

Ligamentous examination **negative** for instability.

Negative Phalen's, Durkin's, and Tinel's for carpal tunnel

No signs of triggering of the thumb, index finger, middle finger, ring finger, or small finger, at the A1 pulley.

Positive Finkelstein's test

Positive thumb grind / distraction test at CMC joint

Sensation intact to light touch to the first dorsal web space of the hand, volar pad of the index finger, volar pad of the small finger.

Allen test positive

Hands are warm and well perfused with 2+ radial artery pulses bilaterally.

WRIST RANGE OF MOTION

	RIGHT	LEFT
EXTENSION (N = 60)		
FLEXION (N = 60)		
RADIAL DEVIATION (N = 20)		
ULNAR DEVIATION (N = 30)		

When testing extremity range of motion, three valid measurements were obtained and the maximum reported herein.

GRIP STRENGTH (lbs)

	RIGHT	LEFT
--	-------	------

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1		
2		
3		

2-POINT Discrimination:

Millimeters

Right volar pad Index finger	
Left volar pad Index finger	

Digital range of motion

	RIGHT	LEFT
Thumb		
MCP		
IP		
Index		
MCP		
PIP		
DIP		
Long		
MCP		
PIP		
DIP		
Ring		
MCP		
PIP		
DIP		
Small		
MCP		
PIP		
DIP		

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LOWER EXTREMITIES

HIP

Right hip:

No redness, swelling, or bruising

No surgical scars

No tenderness to palpation at greater trochanter

No tenderness at anterior hip joint

Negative log-roll

Negative Stinchfield

No pain on flexion adduction internal rotator

No pain on flexion abduction external rotation

Intact strength in gluteus medius

Left hip:

No redness, swelling, or bruising

No surgical scars

No tenderness to palpation at greater trochanter

No tenderness at anterior hip joint

Negative log-roll

Negative Stinchfield

No pain on flexion adduction internal rotator

No pain on flexion abduction external rotation

Intact strength in gluteus medius

HIP RANGE OF MOTION

	RIGHT	LEFT
FLEXION (w/knee flexed) (N = 100)		
EXTENSION (N = -10)		
INTERNAL ROTATION (prone) (N = 25)		
EXTERNAL ROTATION (prone) (N = 35)		
ABDUCTION (N = 20)		
ADDUCTION (-)		
ABDUCTION CONTRACTURE		

GAIT EVALUATION:

The applicant has a **normal**-based gait with **normal** cadence and **normal** stride length, bilaterally.

There is no asymmetry and no antalgic component.

No Trendelenburg lurch or limp.

Normal foot progression angle bilaterally.

Limb length inequality measurement :

Right:

Left:

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KNEE

Right knee:

No effusion

No surgical scars

No signs of quadriceps atrophy

Negative patellar apprehension sign.

Negative patellar grind test.

No evidence of lateral tilt or patellar subluxation.

Stable to varus/valgus stress at 0/30 degrees

Negative Lachman, anterior drawer, posterior drawer

Negative pivot shift exam

Negative rotational dial test at 0°, 30°, and 90° of flexion of the knee.

Tenderness to palpation medial joint line

Tenderness to palpation over Pes Anserine Bursa

Tenderness to palpation lateral joint line

There is no tenderness to palpation over the medial collateral ligament or the lateral collateral ligament period

Tenderness to palpation anterior extensor mechanism, able to straight leg raise against resistance.

Negative McMurray's test medially and laterally for pain with no click

Left knee:

No effusion

No surgical scars

No signs of quadriceps atrophy

Negative patellar apprehension sign.

Negative patellar grind test.

No evidence of lateral tilt or patellar subluxation.

Stable to varus/valgus stress at 0/30 degrees

Negative Lachman, anterior drawer, posterior drawer

Negative pivot shift exam

Negative rotational dial test at 0°, 30°, and 90° of flexion of the knee.

Tenderness to palpation medial joint line

Tenderness to palpation over Pes Anserine Bursa

Tenderness to palpation lateral joint line

There is no tenderness to palpation over the medial collateral ligament or the lateral collateral ligament period

Tenderness to palpation anterior extensor mechanism, able to straight leg raise against resistance.

Negative McMurray's test medially and laterally for pain with no click

KNEE RANGE OF MOTION

	RIGHT	LEFT
EXTENSION (N = 0)		
FLEXION (N = 110)		

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LEG / ANKLE / FOOT

Right lower extremity:

Hindfoot in neutral alignment

2+ DP/PT pulses

Sensation intact to light touch medial, lateral, dorsal, plantar, first dorsal webspace

Intact dorsiflexion, plantarflexion, inversion, eversion

No effusion

Negative anterior drawer, CFL stress

No edema or tenderness about peroneal tendons

No edema or tenderness about posterior tibial tendon

No edema or tenderness about Achilles tendon

Nontender to palpation sinus tarsi

Nontender to palpation midfoot

Nontender to palpation origin of the plantar fascia

Left lower extremity:

Hindfoot in neutral alignment

2+ DP/PT pulses

Sensation intact to light touch medial, lateral, dorsal, plantar, first dorsal webspace

Intact dorsiflexion, plantarflexion, inversion, eversion

No effusion

Negative anterior drawer, CFL stress

No edema or tenderness about peroneal tendons

No edema or tenderness about posterior tibial tendon

No edema or tenderness about Achilles tendon

Nontender to palpation sinus tarsi

Nontender to palpation midfoot

Nontender to palpation origin of the plantar fascia

FOOT AND ANKLE RANGE OF MOTION

	RIGHT	LEFT
PLANTARFLEXION (N = 25)		
DORSIFLEXION (N = 15)		
INVERSION		
EVERSION		
ADDUCTION		
ABDUCTION		
GT (Max -Baseline) Extension		
GT (Max -Baseline) Flexion		

When testing extremity range of motion, three valid measurements were obtained and the maximum reported herein.

Bilateral lower extremities are warm and well perfused with 2+ pedal pulses.

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DTR'S

	RIGHT	LEFT
Patellar	2+	2+
Achilles	2+	2+

DIAGNOSTIC IMPRESSION

1.

DISCUSSION

According to the applicant's history on today's evaluation, the applicant is claiming a specific injury/continuous trauma to ***

According to a cover letter dated _____ and signed by _____, the applicant is alleging specific injury on _____/cumulative trauma from ____ to _____. (Add any additional information from cover letters received that is pertinent to this particular evaluation)

(If TWO [1 from AA, 1 from DA/ADJ cover letters, repeat the above])

As an orthopaedic surgeon, I will comment only on musculoskeletal injuries and will defer on nonorthopaedic injuries to the appropriate specialist.

DISCUSSION OF CREDIBILITY OF APPLICANT:

The applicant was forthcoming and appeared sincere in discussing the details related to the injury. There was no evidence of symptom exaggeration. The account of injury and treatment was consistent with that provided within the medical records available for review. There were, however, some inconsistencies which warrant special mention:

Given these considerations, I deem the applicant to be of *** credibility and this finding will be taken into account in assimilating the subjective elements of his history in the determination of causation, impairment, permanent disability, and apportionment.

OPINION ON CAUSATION

A finding of industrial causation is predicated on a credible applicant, a plausible mechanism of injury that is consistently substantiated in the medical records, and corresponds to clinical and diagnostic findings, and appropriate participation in, and response to, available medical treatment as indicated in the medical records. The burden of presenting substantive information both as it pertains to their mechanism of injury and their evolution of symptoms in order to build a case for industrial causation rests on the applicant.

The applicant is claiming a specific injury/continuous trauma to [***body regions***]

IF SPECIFIC INJURY:

I have reviewed the medical evidence presented and considered the history obtained from the applicant. It is within reasonable medical probability the applicant sustained a specific injury to to [body part(s)] on **, while employed by ** as a **. (Provide description of mechanism of

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trauma and how it correlates with the objective findings on examination and diagnoses. Reference the medical evidence you have received that corroborates your opinion on causation [typically the initial report following the injury is the best source – it establishes how the injury took place and what was initially examined]. If there is medical evidence of a prior injury or the applicant discloses one, make note of that, indicating it could be an apportionable issue if medical records are provided and relevant. If diagnostic imaging reveals degenerative changes, address this. Were the degenerative changes pre-existing and caused to become symptomatic? Would the degenerative changes result in ratable impairment barring the specific injury?

IF CONTINUOUS TRAUMA:

I have reviewed the medical evidence presented and considered the history obtained from the applicant. It is within reasonable medical probability the applicant sustained a continuous trauma to [body part(s)] which spans the applicant's employment with ** from ** to **. ***CT claims are filed as a span of one year, but it's important to explain it was the applicant's entire period of employment that contributed to the overall wear and tear sustained.*** (Provide description of daily regular physical demands that would contribute to the applicant's current objective findings on examination. If diagnostic imaging is provided, note whether or not the daily physical demands could have been contributory to the overall wear and tear of the body region in question. Reference any medical evidence during the period of the CT (employment) that corroborates the applicant's history of when symptoms developed. If there is medical evidence of a prior injury or the applicant discloses one, make note of that, indicating it could be an apportionable issue if medical records are provided and relevant.

*****IF NO CAUSATION*****

Since I am unable to find industrial causation for the applicant's claim of injury arising out of and occurring in the course of his / her employment with ... , discussion of disability status, permanent disability, whole person impairment, apportionment and future medical care is moot. However, I acknowledge that the ultimate determination of causation rests with the Trier of Fact. As such, if a Trier of Fact deems this injury to rise to the level of industrial injury, a supplemental report may be requested in which I would be able to provide my conclusions regarding the aforementioned points.

I believe I have responded to all questions posed to me. If there are any additional questions, please do not hesitate to contact me.

*****IF NO CAUSATION*****

DISABILITY STATUS

It is my medical opinion that all periods of temporary disability noted in the medical records and summarized above were appropriate for and commensurate with the applicant's injuries.

I defer determination of the applicant's candidacy to continue working with modified duties to the primary treating physician. A final discussion of permanent disability, impairment, and apportionment will be deferred until the applicant is at maximum medical improvement.
[PLEASE DELETE IF APPLICANT IS AT MMI/P&S]

*****IF NO MEDICAL RECORDS FOR REVIEW*****

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In order to make a determination of the applicant's disability status, permanent impairment, permanent disability, future work restrictions, and need for future medical care I am requesting all medical records pertaining to this case be submitted for review and discussion in a supplemental report.

In addition, I recommend the following studies:

IF NO MEDICAL RECORDS FOR REVIEW

PERMANENT DISABILITY

***If MMI/P&S, provide permanent work restrictions.

WHOLE PERSON IMPAIRMENT

Impairment is calculated using American Medical Association Guides to the Evaluation of Permanent Impairment (5th Edition).

I do find this impairment rating to be an accurate reflection of the permanent disability in accordance with Almaraz/Guzman.

APPORTIONMENT ACCORDING TO SB 899/LC 4663

Having taken effect on April 19, 2004, SB 899 mandated all discussion on apportionment was to be based upon causation. The new statutes in particular were Labor Code 4663 and 4664, with LC4663 indicating the physician shall make an apportionment determination by finding what approximate percentage of permanent disability was caused by the direct result of injury arising out of and occurring in the course of employment and what approximate percentage of permanent disability was caused by other factors both before and subsequent to the industrial injury including, such as prior injuries, co-morbidities that would predispose to the current disability and underlying nonindustrial conditions contributing to the current level of disability. LC4664 further states if an injured worker has received a prior aware of permanent impairment, it shall be conclusively presumed that the prior permanent impairment existed at the time of any subsequent industrial trauma. In addition, case laws Escobedo, Gatten and Benson have further defined the burden on the evaluator in formulating an opinion on apportionment, when applicable.

Taking into consideration Labor Codes 4663 and 4664, the above referenced case law, the medical history, physical examination, and results of diagnostic studies, all of which combine to meet the standard of substantial medical evidence, it is my medical opinion that **% of the permanent impairment, referable to the **, is a direct result of the specific injury that occurred on **, with **% due to nonindustrial causation. (if you find apportionment, you must provide medical evidence to substantiate the apportionment.)

FUTURE MEDICAL CARE

With regard to the [***body region***], the applicant should engage in a home exercise program including spine range of motion, core strengthening, paraspinal muscle strengthening, and use of heating packs to alleviate muscle spasm. Over the counter medication may be utilized for pain.

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Should the applicant experience flare-ups or exacerbations then medical re-evaluation, physical therapy, chiropractic treatment, and/or injections may be performed as deemed necessary. The applicant's condition does not require surgical treatment in the foreseeable future.

With regard to the [***body region***], the applicant should engage in a home exercise program including joint range of motion, strengthening, and use of heating packs to alleviate muscle spasm. Over the counter medication may be utilized for pain. Should the applicant experience flare-ups or exacerbations then medical re-evaluation, physical therapy, chiropractic treatment, and/or injections may be performed as deemed necessary. The applicant's condition does not require surgical treatment in the foreseeable future.

The applicant's [***body region***] condition does not require any additional medical care in the foreseeable future. Should a flare up or aggravation occur, future care should be apportioned to the causative agent implicated in the exacerbation or aggravation.

ABILITY TO RETURN TO WORK:

I defer determination of the applicant's candidacy to continue working with modified duties to the primary treating physician.

-OR-

The applicant may return to work within the restrictions described above. If the employer is unable to offer a suitable modified position within the described restrictions, then the applicant would be eligible for Supplemental Job Displacement Benefits.

RE: +Applicant last name, first name

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SPECIAL COMMENTARY

The above responses and opinions are based on reasonable medical probabilities, as viewed from the perspective of available documentation and information submitted to this evaluator, including the applicant's direct anamnesis. The history was obtained by medical legal historian, **.

Thank you for the opportunity of serving as the qualified medical evaluator, in the specialty of Orthopaedic surgery, for this most interesting case and condition.

Sincerely,

Steven C. Bast, M.D., Q.M.E.
Qualified Medical Evaluator

Attachments: Appendix B Declaration

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APPENDIX B - DECLARATION

Pursuant to AB 1300, LC Sec. 5703, I have not violated Labor Code section 139.3, and the contents of the report and bill are true and correct to the best of my knowledge. This statement is made under penalty of perjury.

Pursuant to CCR 35.5 (h), I declare under penalty of perjury that I did not discriminate in any way against the parties to the action or the injured worker in the evaluation process or in the content of this report.

I declare under penalty of perjury that the information contained in this report and its attachments, if any, is true and correct to the best of my knowledge and belief, except as to information that I have indicated I received from others. As to that information, I declare under penalty of perjury that the information accurately describes the information provided to me and, except as noted herein, that I believe it to be true.

DATE OF REPORT: +

Dated this + day of + 2025, at Los Angeles County, California.

Steven Bast

Steven C. Bast, M.D., Q.M.E.

Qualified Medical Evaluator